WEAVE

CENTRE FOR INTEGRATIVE PSYCHIATRY

WEAVE PSYCHOTHERAPY — VOL. 12

Landmark Papers & Trials

The Evidence Base — Key RCTs and Meta-Analyses in Psychotherapy

Dodo Bird · NIMH TDCRP · Linehan 1991 · Giesen-Bloo 2006 · Shedler 2010 · Common Factors

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Deep Study

Landmark Papers & Trials — Weave Psychotherapy Vol. 12

D1: Landmark Papers & Trials in Psychotherapy — Deep Study

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1. THE DODO BIRD VERDICT AND COMMON FACTORS

1.1 Luborsky, Singer & Luborsky (1975) — "Everybody Has Won and All Must Have Prizes"

This paper is the origin of the "Dodo bird verdict" in psychotherapy research. Luborsky and colleagues reviewed comparative outcome studies and concluded that different forms of psychotherapy produce roughly equivalent outcomes. The title references the Dodo bird from Alice in Wonderland, who declares after a race that "everybody has won and all must have prizes."

Design: Narrative review of comparative outcome studies

Key finding: No consistent differences in efficacy between bona fide psychotherapies

Clinical implication: The factors common to all therapies (alliance, empathy, expectation) may matter more than technique-specific ingredients

> EXAM PEARL
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> The Dodo bird verdict does NOT claim that therapy is ineffective or that all therapies are equally good for all conditions. It claims that when two bona fide treatments are compared head-to-head for the same condition, differences in outcome are typically small or absent. This is the most misunderstood finding in psychotherapy research.

1.2 Lambert (1992) — The Pie Chart of Outcome Variance

Michael Lambert proposed a model of what accounts for therapeutic change, based on his review of outcome research. His framework became one of the most cited in psychotherapy training.

The Four Factors:

Factor	Variance Explained	Description
Extratherapeutic change	40%	Client factors, spontaneous remission, life events
Therapeutic relationship	30%	Alliance, empathy, warmth, positive regard
Expectancy (placebo)	15%	Hope, credibility of treatment rationale
Technique	15%	Specific model-driven interventions

Clinical implication: The relationship accounts for roughly twice the variance of technique. Training that focuses exclusively on technique while neglecting relational skills is misallocated.

EXAM PEARL

Lambert's 40/30/15/15 split is the single most frequently examined breakdown of therapeutic outcome factors. The exact percentages have been debated, but the rank order is consistent across re-analyses: client/extratherapeutic factors > relationship > expectancy >= technique.

EXAM STRATEGY

When asked "what is the most important factor in psychotherapy outcome," the answer is extratherapeutic/client factors (40%). When asked "what is the most important therapist-delivered factor," the answer is the therapeutic relationship (30%).

1.3 Wampold (2001, 2015) — The Great Psychotherapy Debate

Bruce Wampold's book and subsequent meta-analytic work provided the most rigorous quantitative test of the Dodo bird verdict. He introduced the distinction between the **medical model** (specific ingredients cure specific disorders) and the **contextual model** (common factors account for outcomes).

Design: Meta-analyses of comparative outcome studies

Key findings:

- The overall effect size of psychotherapy vs no treatment: d = 0.80 (large)
- The effect size of differences between bona fide therapies: d = 0.00–0.20 (negligible to small)
- Therapist effects (variability between therapists within the same treatment) account for 5–9% of outcome variance — larger than the difference between treatments
- Alliance accounts for approximately r = 0.28 of outcome variance (medium effect)

CLINICAL ANCHOR

Wampold's work means that who delivers the therapy matters more than which therapy is delivered. A skilled CBT therapist and a skilled psychodynamic therapist will get comparable outcomes. A poor therapist delivering an "evidence-based" protocol will get poor outcomes. This has direct implications for training, supervision, and quality assurance.

EXAM PEARL

Wampold's effect size for the overall benefit of psychotherapy (d = 0.80) is comparable to the effect of many medical interventions. Psychotherapy is not a "soft" intervention — its effect size exceeds that of many medications for common conditions.

2. CBT LANDMARK TRIALS

2.1 Rush, Beck, Kovacs & Hollon (1977) — The First CBT vs Pharmacotherapy Trial

This is the trial that put CBT on the map. Aaron Beck's cognitive therapy was tested against imipramine (then the gold-standard antidepressant) for outpatient depression.

Design: RCT

Sample: N = 41 outpatients with major depression

Intervention: Cognitive therapy (max 20 sessions over 12 weeks) vs imipramine

Key findings:

- Cognitive therapy showed significantly greater improvement on self-report (BDI) and clinician-rated (HRSD) measures
- 79% of CT group showed marked improvement vs 23% of imipramine group
- CT group had lower dropout rate (5% vs 32%)

Clinical implication: A psychological treatment could match or exceed pharmacotherapy for depression. This was revolutionary in an era when depression was considered a purely biological illness requiring medication.

> EXAM PEARL
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> The Rush et al. (1977) trial has been criticised for its small sample, non-blind design, and possible inadequate pharmacotherapy dosing. But its historical significance is enormous — it launched the entire CBT for depression research programme.

2.2 Elkin et al. (1989) — NIMH Treatment of Depression Collaborative Research Program (TDCRP)

The TDCRP was the first large-scale, multi-site, federally funded trial comparing psychotherapies head-to-head with pharmacotherapy and placebo. It remains one of the most discussed trials in psychotherapy research.

Design: Multi-site RCT, 4 arms

Sample: N = 250 outpatients with major depression

Arms: CBT vs IPT vs imipramine + clinical management vs placebo + clinical management

Duration: 16 weeks

Key findings:

- For the full sample, all active treatments outperformed placebo, but differences between active treatments were not significant
- For more severely depressed patients (HRSD >= 20): imipramine and IPT outperformed placebo; CBT did not significantly differ from placebo
- General pattern: imipramine >= IPT >= CBT >= placebo (but most comparisons non-significant)
- Recovery rates: imipramine 57%, IPT 55%, CBT 51%, placebo 29%

EXAM PEARL

The TDCRP is frequently cited as evidence that CBT is less effective for severe depression. This interpretation is contested: CBT performance varied dramatically across sites, suggesting therapist competence issues rather than a limitation of the model. Site 2 (University of Oklahoma), where CBT therapist competence was rated lowest, dragged down the CBT results.

CLINICAL ANCHOR

The TDCRP taught the field three lasting lessons: (1) multi-site trials require rigorous therapist training and monitoring; (2) severity moderates treatment response; and (3) the placebo response rate in depression trials is substantial (~29%).

2.3 DeRubeis et al. (2005) — CBT vs Medications for Moderate-to-Severe Depression

This trial directly addressed the TDCRP criticism that CBT does not work for severe depression.

Design: RCT, 2 sites (University of Pennsylvania, Vanderbilt)

Sample: N = 240 outpatients with moderate-to-severe MDD (HRSD >= 20)

Arms: Cognitive therapy vs paroxetine vs placebo (8 weeks, then active arms continued to 16 weeks)

Key findings:

- At 8 weeks: both CT and paroxetine significantly outperformed placebo
- At 16 weeks: CT and paroxetine showed equivalent response rates (58% vs 58%)
- Site moderated results — Penn (Beck's institution) had better CT outcomes

Clinical implication: CBT is effective for moderate-to-severe depression, directly refuting the TDCRP conclusion.

> EXAM PEARL
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> DeRubeis et al. (2005) is the definitive rebuttal to "CBT doesn't work for severe depression." The response rates were identical to medication at 58% each. Know this trial and the TDCRP as a pair — examiners love this comparison.

2.4 Hollon et al. (2005) — The Enduring Effects of Cognitive Therapy

This is the companion paper to DeRubeis et al. (2005), following patients after treatment discontinuation.

Design: Naturalistic follow-up of the DeRubeis (2005) RCT

Sample: Responders from CT (n = 52) vs responders from paroxetine randomised to continuation medication or placebo withdrawal

Duration: 12–month follow-up after 16–week acute treatment

Key findings:

- Prior CT had significantly lower relapse rate (31%) compared to medication withdrawal (76%, p = 0.004)
- Prior CT was statistically equivalent to continuing medication (31% vs 47%, ns)
- CT's enduring effect was not due to continued therapy — patients were not receiving any treatment during follow-up

> EXAM PEARL
>
> Hollon et al. (2005) established that CBT has an enduring effect that persists after treatment ends. Medication works while you take it; CBT continues to protect after you stop. This is the single most important argument for CBT in depression guidelines.

CLINICAL ANCHOR

The relapse rate after medication discontinuation (76%) is strikingly high. This is why most guidelines now recommend either long-term maintenance medication OR a course of CBT for recurrent depression — not just a single course of antidepressants.

2.5 Clark et al. (2003, 2006) — CBT for Social Anxiety and PTSD

David M. Clark's Oxford group produced two landmark trials that established disorder-specific cognitive therapy as a front-runner for anxiety disorders.

Clark et al. (2003) — Social Anxiety Disorder

Design: RCT

Sample: N = 60 with social phobia

Arms: Individual CT (Clark & Wells model) vs fluoxetine + self-exposure vs placebo

Key findings: CT was significantly superior to both fluoxetine and placebo at post-treatment and follow-up. Effect sizes were very large (d > 1.0 vs placebo).

Ehlers, Clark et al. (2003, 2005) — PTSD

Design: RCT

Sample: N = 85 with chronic PTSD (Ehlers et al., 2005 trial)

Arms: Cognitive therapy (Ehlers & Clark model) vs self-help booklet vs waitlist

Key findings: CT produced large effects (d = 1.27 vs waitlist). 71% achieved reliable and clinically significant change.

EXAM PEARL

Clark's cognitive therapy models for social anxiety and PTSD are now considered among the most effective psychological treatments for any condition. The effect sizes (d > 1.0) exceed typical psychotherapy benchmarks.

3. DBT LANDMARK TRIALS

3.1 Linehan et al. (1991) — The First RCT of DBT for BPD

Marsha Linehan's first RCT was a watershed moment for BPD treatment. Before this trial, the prevailing view was that BPD was untreatable and that patients should be avoided.

Design: RCT

Sample: N = 44 women with BPD and recent parasuicide

Arms: DBT (1 year) vs treatment as usual (TAU)

Key findings:

- DBT significantly reduced frequency of parasuicidal acts (including self-harm and suicide attempts)
- DBT significantly reduced number of inpatient psychiatric days
- DBT had significantly better treatment retention (83% vs 42%)
- No significant difference in depression, hopelessness, or suicidal ideation scores

> EXAM PEARL
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> Linehan (1991) showed DBT reduces self-harm and hospitalisation but NOT depression or suicidal ideation. This distinction matters: DBT targets behavioural dysregulation, not mood per se. When asked about DBT efficacy, specify the outcomes — it reduces the behaviour, not necessarily the distress.

3.2 Linehan et al. (2006) — DBT vs Community Expert Treatment

The major criticism of early DBT trials was that the comparison condition (TAU) was a weak control. This trial addressed that directly.

Design: RCT

Sample: N = 101 women with BPD, recent suicidal or self-injurious behaviour

Arms: DBT (1 year) vs community treatment by experts (CTBE) — therapists nominated as expert BPD treaters

Key findings:

- DBT was significantly superior to CTBE on: suicide attempts (hazard ratio 2.66, fewer in DBT), medical severity of self-harm, emergency department visits, and psychiatric hospitalisations
- DBT had half the rate of suicide attempts compared to CTBE (23% vs 46%)
- Treatment retention was equivalent

> EXAM PEARL
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> Linehan (2006) is the strongest evidence for DBT superiority over genuine expert treatment (not just TAU). The comparison was deliberately rigorous — community experts who treated BPD regularly, not randomly assigned novices.

> CLINICAL ANCHOR
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> A key finding was that even expert clinicians without a structured BPD protocol achieved significantly worse outcomes than DBT. Structure, protocol adherence, and skills training matter — clinical experience alone is insufficient for this population.

3.3 Rathus & Miller (2002) — Adolescent DBT

Design: Quasi-experimental (non-randomised comparison)

Sample: N = 111 suicidal adolescents with BPD features

Arms: DBT-A (12 weeks, adapted for adolescents) vs TAU (12–week supportive-psychodynamic therapy)

Key findings: DBT-A had significantly fewer psychiatric hospitalisations and higher treatment completion. No suicide attempts in the DBT-A group vs 3 in TAU.

> EXAM PEARL
>
> Rathus & Miller (2002) is the foundational study for adolescent DBT (DBT-A). Although not a true RCT, it demonstrated feasibility and safety, leading to the later McCauley et al. (2018) definitive trial showing DBT-A reduces self-harm in adolescents.

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4. PSYCHODYNAMIC LANDMARK TRIALS

4.1 Shedler (2010) — The Efficacy of Psychodynamic Psychotherapy

Jonathan Shedler's American Psychologist paper was a landmark review that challenged the narrative that psychodynamic therapy lacked an evidence base.

Design: Meta-analytic review

Key findings:

- Effect sizes for psychodynamic therapy (d = 0.97 for targeted outcomes, d = 0.73 for general symptom improvement) are as large as those reported for other evidence-based therapies
- Uniquely, patients who receive psychodynamic therapy continue to improve after treatment ends (the "sleeper effect")
- Non-psychodynamic therapies may work, in part, because they contain psychodynamic processes (e.g., working alliance, exploration of affect, identification of recurring patterns)

EXAM PEARL

Shedler (2010) showed that the effect size for psychodynamic therapy (d = 0.97) matches CBT benchmarks. The "sleeper effect" — continuing improvement after termination — was unique to psychodynamic therapy and was not seen with CBT or medication.

EXAM STRATEGY

If asked "is psychodynamic therapy evidence-based?", cite Shedler (2010) and Leichsenring & Rabung (2008, 2011). The evidence base is smaller than CBT's, but the effect sizes are comparable.

4.2 Leichsenring & Rabung (2008, 2011) — Long-Term Psychodynamic Psychotherapy Meta-Analyses

Design: Meta-analyses of RCTs and controlled trials of long-term psychodynamic psychotherapy (LTPP, defined as > 1 year or > 50 sessions)

Samples: 2008 meta-analysis: 23 studies (N = 1,053). 2011 update: 10 RCTs

Key findings:

- LTPP showed large effect sizes for overall effectiveness (d = 0.96, 2008), complex mental disorders (d = 1.03, personality disorders: d = 0.97)
- LTPP was significantly more effective than shorter forms of therapy for complex/comorbid presentations
- The 2011 update confirmed findings with stricter RCT-only inclusion: LTPP superior to less intensive interventions (d = 0.44)

CLINICAL ANCHOR

"Complex mental disorders" in these meta-analyses means personality disorders, chronic depression, multiple comorbidities, and treatment-resistant presentations. The finding that LTPP is superior for complexity, not for simple single-diagnosis cases, is clinically important — it guides appropriate treatment matching.

4.3 Bateman & Fonagy (1999, 2001, 2008) — Mentalisation-Based Treatment for BPD

Anthony Bateman and Peter Fonagy developed MBT specifically for BPD and tested it in a landmark partial hospitalisation trial.

Bateman & Fonagy (1999) — Original Trial

Design: RCT

Sample: N = 38 with BPD, partial hospitalisation setting

Arms: MBT-based partial hospitalisation (18 months) vs standard psychiatric care

Key findings: MBT significantly reduced self-harm, suicide attempts, inpatient days, depression, anxiety, and interpersonal distress. Effects appeared after 6 months and continued to increase.

Bateman & Fonagy (2001, 2008) — Follow-Up Studies

18-month follow-up (2001): MBT gains were maintained and continued to improve, while the control group showed no change.

8-year follow-up (2008): MBT group maintained significantly better functioning. Suicidality and service use remained lower. 13% of MBT patients still met BPD criteria vs 87% of control.

> EXAM PEARL
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> Bateman & Fonagy's 8-year follow-up (2008) is the longest follow-up in any BPD treatment trial. Only 13% of MBT patients still met BPD criteria at 8 years vs 87% of controls. This is the strongest longitudinal evidence for any BPD treatment.

4.4 Clarkin et al. (2007) — TFP vs DBT vs Supportive Therapy for BPD

Design: RCT

Sample: N = 90 women with BPD

Arms: Transference-Focused Psychotherapy (TFP, 1 year) vs DBT (1 year) vs psychodynamic supportive therapy (1 year)

Key findings:

- All three treatments produced significant improvement across multiple domains
- TFP and DBT significantly reduced suicidality; supportive therapy did not
- Only TFP significantly improved reflective functioning (mentalisation) and attachment organisation
- DBT uniquely improved impulsivity-related behaviours

> EXAM PEARL
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> Clarkin (2007) is the only trial comparing TFP, DBT, and supportive therapy head-to-head. TFP was the only treatment that improved reflective functioning and attachment. DBT was the only one that improved impulsivity. Each modality has a distinct mechanism signature.

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5. SCHEMA THERAPY LANDMARK TRIALS

5.1 Giesen-Bloo et al. (2006) — Schema Therapy vs TFP for BPD

This is the largest and most methodologically rigorous trial of any psychotherapy for BPD. It was conducted across four sites in the Netherlands and directly compared two structured, long-term treatments.

Design: Multi-centre RCT

Sample: N = 86 with BPD (44 ST, 42 TFP), 3–year treatment

Arms: Schema therapy (2 sessions/week, 3 years) vs Transference-Focused Psychotherapy (2 sessions/week, 3 years)

Key findings:

- ST showed significantly greater improvement on all primary outcomes (BPD severity, quality of life, psychopathology)
- Recovery rate: ST 45.5% vs TFP 24.4%
- Clinical improvement: ST 65.9% vs TFP 42.9%
- ST had significantly lower dropout: 26.7% vs 50.6% for TFP
- Effect sizes favoured ST from year 1 onwards, with advantages increasing over time

> EXAM PEARL
>
> Giesen-Bloo et al. (2006) produced the highest recovery rate ever reported for BPD in an RCT (45.5%). Know this number. The dropout rate (26.7%) was also notably low for a BPD trial, reflecting ST's emphasis on limited reparenting and alliance.

> CLINICAL ANCHOR
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> The TFP dropout rate (50.6%) was comparable to dropout from unstructured treatment. The ST dropout rate (26.7%) was half that of TFP. For a population notorious for treatment attrition, this is clinically significant. Limited reparenting and the warm therapeutic stance in ST appear to be protective against dropout.

> EXAM STRATEGY
>
> If asked to compare ST, TFP, DBT, and MBT for BPD, know the trial evidence for each: Giesen-Bloo (2006) for ST, Clarkin (2007) for TFP, Linehan (2006) for DBT, Bateman & Fonagy (1999/2008) for MBT. All show efficacy; ST has the highest recovery rate, MBT has the longest follow-up, DBT has the strongest evidence for reducing self-harm behaviour.

5.2 Nadort et al. (2009) — Schema Therapy With and Without Phone Crisis Support

Design: Multi-centre RCT

Sample: N = 62 with BPD

Arms: ST with therapist phone availability between sessions vs ST without phone availability

Key findings:

- Both groups showed large improvements in BPD symptoms, with recovery rates comparable to Giesen-Bloo et al.
- No significant differences between groups — phone crisis support did not add benefit beyond ST alone
- This demonstrated ST's effectiveness without requiring 24/7 therapist availability (a criticism often levelled at DBT)

> EXAM PEARL
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> Nadort et al. (2009) showed that ST's efficacy does not depend on between-session phone contact, making it more practical and less burdensome than DBT for therapists. ST works through the therapeutic relationship in-session, not through crisis coaching.

5.3 Bamelis et al. (2014) — Schema Therapy for Cluster C Personality Disorders

Design: Multi-centre RCT

Sample: N = 323 with Cluster C PDs (avoidant, dependent, obsessive-compulsive), PD NOS, or paranoid PD

Arms: ST (50 sessions) vs Clarification-Oriented Psychotherapy (COP) vs TAU

Key findings:

- ST showed significantly higher recovery rates than COP and TAU (81% vs 60% vs 52% at the 1–year follow-up)
- ST was more cost-effective per quality-adjusted life year (QALY) gained
- ST was effective across all Cluster C subtypes

> EXAM PEARL
>
> Bamelis (2014) is the largest trial of any psychotherapy for personality disorders (N = 323). It established ST's efficacy beyond BPD — specifically for Cluster C PDs. The 81% recovery rate for ST is remarkable for personality disorders.

6. EXPOSURE THERAPY AND EMDR

6.1 Foa, Dancu, Hembree et al. (1999, 2005) — Prolonged Exposure for PTSD

Edna Foa's work on Prolonged Exposure (PE) is the foundation of trauma-focused treatment.

Foa et al. (1999)

Design: RCT

Sample: N = 96 female sexual assault survivors with chronic PTSD

Arms: PE vs stress inoculation training (SIT) vs combined PE + SIT vs waitlist

Key findings:

- All active treatments significantly outperformed waitlist
- PE alone showed the best overall outcome at follow-up
- The combined treatment was NOT superior to PE alone — a counterintuitive finding

Foa et al. (2005)

Design: RCT

Sample: N = 179 women with PTSD

Arms: PE vs PE + cognitive restructuring vs waitlist

Key findings: PE alone and PE + CR were equally effective. Adding cognitive restructuring did not improve outcomes beyond exposure alone.

> EXAM PEARL
>
> Foa's work consistently shows that exposure is the critical ingredient in PTSD treatment. Adding cognitive restructuring, stress inoculation, or other components does not significantly improve outcomes beyond PE alone. Exposure is necessary and may be sufficient.

6.2 Resick et al. (2002) — Cognitive Processing Therapy vs Prolonged Exposure

Design: RCT

Sample: N = 171 female sexual assault survivors with PTSD

Arms: CPT (12 sessions) vs PE (9 sessions) vs minimal attention control

Key findings:

- Both CPT and PE were significantly superior to control
- CPT and PE showed equivalent outcomes on PTSD severity
- Both maintained gains at 9-month follow-up
- CPT showed a slight (non-significant) advantage on guilt cognitions

> CLINICAL ANCHOR
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> CPT and PE are the two front-line trauma treatments recommended by the VA/DoD guidelines. For clinical matching: PE is preferred when avoidance of trauma memories is the primary maintaining factor. CPT is preferred when "stuck points" (maladaptive beliefs about the trauma) are prominent.

6.3 Shapiro (1989) — The Original EMDR Study

Design: RCT

Sample: N = 22 with traumatic memories (rape, molestation, Vietnam veterans)

Arms: EMDR (single session) vs modified flooding with saccades

Key findings: EMDR produced significant reductions in subjective distress (SUD scores) and increases in positive cognition validity (VOC scores) after a single session.

> EXAM PEARL
>
> Shapiro (1989) has been heavily criticised for: small sample, therapist allegiance (Shapiro administered all treatments), non-blind assessment, and inadequate comparison condition. However, it generated a research programme that has since accumulated strong evidence. Know the criticisms but also know the subsequent positive trials.

6.4 Bisson et al. (2007) — Cochrane Review of Psychological Treatments for PTSD

Design: Cochrane systematic review and meta-analysis

Sample: 38 RCTs

Key findings:

- Trauma-focused CBT (including PE and CPT) and EMDR were the only treatments with sufficient evidence for recommendation
- Both TF-CBT and EMDR significantly outperformed waitlist and non-trauma-focused therapies
- No significant difference between TF-CBT and EMDR
- Non-trauma-focused therapies (supportive counselling, relaxation) showed limited evidence
- Group CBT was less effective than individual CBT

> EXAM PEARL
>
> The Cochrane review (Bisson, 2007) gave NICE and WHO their evidence base for recommending TF-CBT and EMDR as first-line PTSD treatments. The specific finding that trauma-focused approaches outperform non-trauma-focused ones is critical — supportive counselling is not adequate for PTSD.

7. FAMILY AND COUPLES THERAPY

7.1 Gottman (1999) — The Mathematics of Divorce Prediction

John Gottman's research programme, culminating in his book "The Seven Principles for Making Marriage Work," used observational coding of marital interactions to predict divorce with remarkable accuracy.

Design: Prospective longitudinal studies with observational coding (Specific Affect Coding System, SPAFF)

Samples: Multiple cohorts totalling over 3,000 couples

Key findings:

- The "Four Horsemen" — criticism, contempt, defensiveness, and stonewalling — predicted divorce with 93.6% accuracy in a 6-year follow-up
- The ratio of positive to negative interactions during conflict was the strongest predictor. Stable couples showed a 5:1 ratio; divorce-bound couples showed 0.8:1
- Contempt was the single most toxic behaviour, predicting not only divorce but also physical illness in the recipient
- Physiological arousal (heart rate, skin conductance) during conflict predicted relationship deterioration

> EXAM PEARL
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> Gottman's Four Horsemen of the Apocalypse: Criticism, Contempt, Defensiveness, Stonewalling. Contempt is the most damaging. The 5:1 positive-to-negative ratio during conflict is the stability threshold. These are among the most asked questions in couples therapy examinations.

7.2 Johnson, Hunsley, Greenberg & Schindler (1999) — EFT for Couples

Design: Meta-analysis

Sample: 7 studies of Emotionally Focused Couple Therapy (EFT-C)

Key findings:

- Large effect size: d = 1.30 (post-treatment change)
- 70–73% of couples moved to recovery; 90% showed significant improvement
- Gains were maintained at follow-up (2 years in some studies)
- Moderators: greater initial distress predicted larger gains; EFT was effective across different presenting problems

> EXAM PEARL
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> EFT-C's effect size of d = 1.30 is the largest reported for any couples therapy approach. Know that EFT-C (Johnson) works through accessing attachment emotions and restructuring interactional cycles — it is NOT the same as Greenberg's EFT for individuals, though they share theoretical roots.

7.3 Leff et al. (2000) — Family Intervention for Schizophrenia

Design: Meta-analysis and critical review of family intervention trials

Key findings:

- Family interventions (psychoeducation, communication training, problem-solving) reduced relapse rates in schizophrenia by approximately 50% compared to standard care
- 9–month relapse: ~25% with family intervention vs ~50% without
- Family interventions reduced high expressed emotion (EE), the strongest psychosocial predictor of relapse

- Most effective when: lasting > 6 months, including the patient, and providing concrete coping skills

> CLINICAL ANCHOR
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> High expressed emotion (criticism, hostility, emotional over-involvement) in family members is one of the most replicated predictors of relapse in schizophrenia. Family interventions targeting EE are now recommended in every major schizophrenia guideline (NICE, APA, PORT). The evidence is strong enough that NOT offering family intervention represents a gap in care.

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8. ACT AND MBCT

8.1 Teasdale et al. (2000) — MBCT Prevents Depression Relapse

Design: Multi-site RCT

Sample: N = 145 with recurrent MDD (>= 3 prior episodes), currently in remission

Arms: MBCT (8–week group programme) + TAU vs TAU alone

Key findings:

- For patients with >= 3 prior episodes (77% of sample): MBCT significantly reduced relapse — 37% relapsed vs 66% in TAU (p = 0.004)
- For patients with only 2 prior episodes: MBCT did NOT reduce relapse
- MBCT's mechanism: decentering from depressive cognitions, breaking the reactivation of ruminative processing patterns

> EXAM PEARL
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> Teasdale (2000) showed MBCT prevents relapse in patients with >= 3 prior episodes of depression, NOT in those with only 2 episodes. This specificity is crucial: MBCT works by disrupting cognitive reactivity, which is more established in highly recurrent depression. The 3+ episodes threshold is the standard clinical indication.

8.2 Kuyken et al. (2015) — MBCT vs Maintenance Antidepressants

Design: Multi-centre RCT (the PREVENT trial)

Sample: N = 424 with recurrent MDD (>= 3 episodes), on maintenance antidepressants

Arms: MBCT with support to taper antidepressants vs maintenance antidepressants

Duration: 24–month follow-up

Key findings:

- MBCT with antidepressant tapering was non-inferior to maintenance antidepressants for preventing depressive relapse over 24 months
- Relapse: MBCT 44% vs maintenance medication 47% (hazard ratio 0.89, ns)
- MBCT was more effective than medication for patients with higher severity of childhood abuse

> EXAM PEARL
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> Kuyken (2015) is the definitive MBCT trial. It established MBCT as a viable alternative to long-term antidepressant maintenance. The finding that childhood abuse survivors benefited more from MBCT than medication suggests MBCT may particularly help patients whose depression is maintained by ruminative processing of early adversity.

> EXAM STRATEGY
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> Know the triad of MBCT evidence: Teasdale (2000) = prevention, Ma & Teasdale (2004) = replication, Kuyken (2015) = non-inferiority to medication.

8.3 A-Tjak et al. (2015) — ACT Meta-Analysis

Design: Meta-analysis of RCTs

Sample: 39 RCTs across multiple conditions

Key findings:

- ACT outperformed control conditions (waitlist, placebo, TAU) with a small-to-medium effect: g = 0.57 (95% CI 0.44–0.70)
- ACT was NOT significantly different from established treatments (CBT): g = 0.06 (ns)
- ACT was effective for chronic pain (g = 0.62), anxiety disorders (g = 0.51), depression (g = 0.56), and substance use (g = 0.45)
- Process measures: psychological flexibility mediated outcomes across studies

> EXAM PEARL
>
> A-Tjak (2015) is the benchmark ACT meta-analysis. ACT works — it outperforms control conditions with medium effect sizes. But it does NOT outperform traditional CBT. The claim that ACT is "better" than CBT is not supported; the claim that ACT is an effective, evidence-based alternative is.

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9. COMPARATIVE AND INTEGRATION STUDIES

9.1 Cuijpers et al. (2008, 2019) — Meta-Analyses of Psychotherapy for Depression

Pim Cuijpers and his group at VU Amsterdam have produced the most comprehensive meta-analytic programme in psychotherapy research.

Cuijpers et al. (2008)

Design: Meta-analysis

Sample: 53 RCTs of psychotherapy for adult depression

Key findings:

- Overall effect size: d = 0.67 (moderate to large)

- CBT, behavioural activation, IPT, psychodynamic therapy, and problem-solving therapy all showed significant effects
- No significant differences between bona fide therapies (supporting the Dodo bird verdict)
- When corrected for publication bias, the effect size dropped to d = 0.42

> EXAM PEARL
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> Cuijpers (2008) estimated that publication bias inflates psychotherapy effect sizes by approximately 25%. The corrected effect size (d = 0.42) is still clinically meaningful but more modest than the uncorrected estimate. Always consider publication bias when interpreting psychotherapy meta-analyses.

Cuijpers et al. (2019) — Updated Network Meta-Analysis

Design: Network meta-analysis

Sample: 331 RCTs (N > 34,000)

Key findings:

- All seven types of psychotherapy were more effective than controls for adult depression
- Behavioural activation, CBT, IPT, problem-solving therapy, psychodynamic therapy, life review, and third-wave approaches all showed significant effects
- The effects did not differ significantly between types (again supporting equivalence)
- Combined psychotherapy + pharmacotherapy was more effective than either alone

> CLINICAL ANCHOR
>
> The consistent finding across Cuijpers' meta-analyses is that combination treatment (therapy + medication) outperforms either alone. For moderate-to-severe depression, the evidence strongly supports combination. This is reflected in NICE, APA, and CANMAT guidelines.

9.2 Steinert et al. (2017) — Psychodynamic Therapy vs CBT

Design: Systematic review and meta-analysis

Sample: 23 RCTs directly comparing psychodynamic therapy with CBT

Key findings:

- No significant difference in primary outcomes between psychodynamic therapy and CBT: g = -0.07 (95% CI -0.21 to 0.08)
- Results were consistent across depression, anxiety, and personality disorders
- Neither approach was consistently superior to the other

> EXAM PEARL
>
> Steinert (2017) is the most cited direct comparison meta-analysis of psychodynamic vs CBT. The effect difference was essentially zero (g = -0.07). Use this to counter the narrative that CBT is "more evidence-based" — it has more trials, but when directly compared, outcomes are equivalent.

9.3 Barlow's Unified Protocol (UP)

The Unified Protocol for Transdiagnostic Treatment of Emotional Disorders (Barlow et al., 2011) represents the integration movement in CBT — treating the shared mechanisms underlying all emotional disorders rather than using disorder-specific protocols.

Core modules: Mindful emotion awareness, cognitive flexibility, countering emotional behaviours (exposure), interoceptive awareness

Key trials:

- Farchione et al. (2012): N = 37, open trial, significant improvement across anxiety disorders (d = 0.52–1.14)
- Barlow et al. (2017): N = 223 RCT, UP vs single-disorder protocols vs waitlist. UP was non-inferior to single-disorder protocols and superior to waitlist

across all anxiety disorders

- Cassiello-Robbins et al. (2020): UP showed equivalent outcomes to disorder-specific CBT with similar treatment gains

> EXAM PEARL
>
> Barlow's Unified Protocol is the most researched transdiagnostic CBT. It treats neuroticism (the shared vulnerability across emotional disorders) rather than disorder-specific symptoms. The non-inferiority finding vs disorder-specific protocols is significant — one protocol can treat comorbid anxiety and mood disorders simultaneously.

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10. PROCESS RESEARCH

10.1 Bordin (1979) — The Working Alliance

Edward Bordin proposed a pantheoretical model of the therapeutic alliance with three components:

1. **Tasks** — agreement on what happens in therapy
2. **Goals** — agreement on desired outcomes
3. **Bond** — mutual trust, liking, and attachment

Impact: The Working Alliance Inventory (WAI), based on Bordin's model, became the most widely used alliance measure. Subsequent meta-analyses (Horvath & Symonds, 1991; Horvath et al., 2011) confirmed a consistent relationship between alliance and outcome: $r = 0.275$ (medium effect), stable across therapies, raters, and time of measurement.

> EXAM PEARL
>
> Bordin's three alliance components: Tasks, Goals, Bond. The alliance-outcome correlation ($r = 0.275$) is one of the most replicated findings in psychotherapy research. It accounts for roughly 7.5% of outcome variance — modest but consistent and significant.

> CLINICAL ANCHOR
>
> The alliance is not just "rapport." It includes agreement on what you are doing (tasks) and why (goals). A therapist who has excellent rapport but uses interventions the patient does not understand or agree with has a poor task alliance — and this predicts poorer outcomes.

10.2 Safran & Muran (2000) — Alliance Rupture and Repair

Jeremy Safran and J. Christopher Muran's programme of research on rupture-repair sequences revolutionised how clinicians understand the therapeutic relationship.

Key concepts:

- **Rupture:** A deterioration or strain in the alliance — can be a confrontation (patient expresses anger, dissatisfaction) or a withdrawal (patient disengages, becomes compliant, changes topic)
- **Repair:** The process of recognising, addressing, and resolving the rupture within the therapy
- **Resolution:** Successful repair predicts better outcome than never having a rupture at all

Research findings:

- Rupture-repair sequences predict positive outcome more strongly than consistently high alliance ratings
- The ability to repair ruptures distinguishes effective from ineffective therapists
- Withdrawal ruptures are harder to detect than confrontation ruptures and are more common

EXAM PEARL

Safran & Muran's key finding: repaired ruptures predict BETTER outcomes than no rupture at all. A therapy that goes smoothly may mean the patient is being compliant rather than engaged. The rupture-repair cycle is itself therapeutic — it provides a corrective emotional experience.

EXAM STRATEGY

There are two types of rupture: confrontation (the patient pushes back) and withdrawal (the patient shuts down). Withdrawal ruptures are more dangerous because they are harder to detect. If a patient suddenly becomes agreeable and compliant after previously being engaged, suspect a withdrawal rupture.

10.3 Kazdin (2007) — Mediators and Mechanisms of Change

Alan Kazdin's landmark paper challenged the field to move beyond "does therapy work?" to "how and why does therapy work?"

Key arguments:

- Establishing that a treatment works (efficacy) is not the same as understanding why it works (mechanism)
- A mediator is a statistical concept (a variable that accounts for the treatment-outcome relationship). A mechanism is a causal process (the actual reason change occurs)
- Most claimed mechanisms in psychotherapy (cognitive change in CBT, insight in psychodynamic therapy, mentalisation in MBT) have not met the criteria for true mechanism demonstration
- Requirements for mechanism evidence: strong association with outcome, specificity to that treatment, temporal precedence (the mechanism changes before the outcome changes), gradient (more mechanism change = more outcome change), and a plausible theoretical account

EXAM PEARL

Kazdin (2007) distinguished mediators from mechanisms. A mediator is a statistical finding (X accounts for the Y-Z relationship). A mechanism requires temporal precedence, specificity, gradient, and plausibility. Very few psychotherapy "mechanisms" have been truly demonstrated — most remain at the mediator level.

CLINICAL ANCHOR

This paper has practical implications. If we do not know how therapy works, we cannot efficiently train therapists, improve protocols, or match patients to treatments. Kazdin's call to study mechanisms has shaped the next generation of psychotherapy research, including the dismantling studies and process-outcome studies that now dominate the field.

MASTER TABLE: LANDMARK TRIALS IN

PSYCHOTHERAPY

Trial	Year	Design	N	Condition	Comparison	Key Finding	Effect Size
Luborsky et al.	1975	Review	Multiple	Various	Various psychotherapies	"Dodo bird verdict" — equivalent outcomes	—
Rush et al.	1977	RCT	41	Depression	CT vs imipramine	CT superior on all measures	—
Bordin	1979	Theory	—	—	—	Working alliance: tasks, goals, bond	r = 0.275
Shapiro	1989	RCT	22	Trauma	EMDR vs flooding variant	EMDR reduced distress rapidly	—
Elkin et al. (TDCRP)	1989	Multi-site RCT	250	Depression	CBT vs IPT vs imipramine vs placebo	All active > placebo; imipramine best for severe	d = 0.37 (vs placebo)

Trial	Year	Design	N	Condition	Comparison	Key Finding	Effect Size
Linehan et al.	1991	RCT	44	BPD	DBT vs TAU	DBT reduced parasuicide + hospitalisations	—
Lambert	1992	Review	—	Various	—	40/30/15/15 outcome variance model	—
Bateman & Fonagy	1999	RCT	38	BPD	MBT partial hospital vs standard care	MBT reduced self-harm, depression, hospitalisations	—
Foa et al.	1999	RCT	96	PTSD	PE vs SIT vs combined vs waitlist	PE alone best at follow-up	—
Gottman	1999	Longitudinal	>3000	Couples	Observational coding	Four Horsemen predict divorce (93.6%)	—

Trial	Year	Design	N	Condition	Comparison	Key Finding	Effect Size
Johnson et al.	1999	Meta-analysis	7 studies	Couples	EFT-C vs controls	EFT-C d = 1.30; 70–73% recovery	d = 1.30
Leff et al.	2000	Meta-analysis	Multiple	Schizophrenia	Family intervention vs standard care	50% relapse reduction	—
Safran & Muran	2000	Process research	Multiple	Various	—	Rupture-repair predicts better outcomes	—
Teasdale et al.	2000	RCT	145	Recurrent MDD	MBCT + TAU vs TAU	MBCT halved relapse for 3+ episodes	—
Wampold	2001	Meta-analysis	Multiple	Various	Between-therapy comparisons	Therapy differences d = 0.00–0.20	d = 0.80 (vs no tx)
Rathus & Miller	2002	Quasi-exp.	111	Adolescent BPD	DBT-A vs TAU	DBT-A reduced hospitalisations, 0 suicide attempts	—

Trial	Year	Design	N	Condition	Comparison	Key Finding	Effect Size
Resick et al.	2002	RCT	171	PTSD	CPT vs PE vs control	CPT = PE; both superior to control	—
Clark et al.	2003	RCT	60	Social anxiety	CT vs fluoxetine vs placebo	CT superior to fluoxetine and placebo	d > 1.0
DeRubeis et al.	2005	RCT	240	Moderate-severe MDD	CT vs paroxetine vs placebo	CT = paroxetine (58% response each)	—
Foa et al.	2005	RCT	179	PTSD	PE vs PE + CR vs waitlist	PE alone = PE + CR	—
Hollon et al.	2005	Follow-up	104	Depression	Prior CT vs medication continuation vs withdrawal	Prior CT relapse 31% vs withdrawal 76%	—
Giesen-Bloo et al.	2006	Multi-site RCT	86	BPD	ST vs TFP (3 years)	ST recovery 45.5% vs TFP 24.4%	—

Trial	Year	Design	N	Condition	Comparison	Key Finding	Effect Size
Linehan et al.	2006	RCT	101	BPD	DBT vs community experts	DBT halved suicide attempts (23% vs 46%)	—
Bisson et al.	2007	Cochrane review	38 RCTs	PTSD	Multiple comparisons	TF-CBT and EMDR recommended; equivalent to each other	—
Clarkin et al.	2007	RCT	90	BPD	TFP vs DBT vs supportive	All improved; TFP improved mentalisation; DBT improved impulsivity	—

Trial	Year	Design	N	Condition	Comparison	Key Finding	Effect Size
Kazdin	2007	Conceptual	—	—	—	Mediators are not mechanisms; criteria for mechanism evidence	—
Cuijpers et al.	2008	Meta-analysis	53 RCTs	Depression	Multiple psychotherapies	d = 0.67 (0.42 corrected for pub bias); no between-therapy differences	d = 0.67
Leichsenring & Rabung	2008	Meta-analysis	23 studies	Complex disorders	LTPP vs shorter therapies	LTPP d = 0.96 for complex presentations	d = 0.96
Bateman & Fonagy	2008	8-year follow-up	38	BPD	MBT vs standard care	13% still met BPD criteria vs 87% control	—

Trial	Year	Design	N	Condition	Comparison	Key Finding	Effect Size
Nadort et al.	2009	RCT	62	BPD	ST + phone vs ST alone	No added benefit of phone support	—
Shedler	2010	Meta-review	Multiple	Various	Psychodynamic vs benchmarks	d = 0.97; "sleeper effect" unique to psychodynamic	d = 0.97
Bamelis et al.	2014	Multi-site RCT	323	Cluster C PD	ST vs COP vs TAU	ST 81% recovery; largest PD trial	—
A-Tjak et al.	2015	Meta-analysis	39 RCTs	Various	ACT vs controls and CBT	ACT g = 0.57 vs controls; g = 0.06 vs CBT (ns)	g = 0.57
Kuyken et al.	2015	RCT	424	Recurrent MDD	MBCT + taper vs maintenance meds	MBCT non-inferior to medication over 24 months	—

Trial	Year	Design	N	Condition	Comparison	Key Finding	Effect Size
Steinert et al.	2017	Meta-analysis	23 RCTs	Various	Psychodynamic vs CBT	No significant difference: g = -0.07	g = -0.07
Cuijpers et al.	2019	Network meta-analysis	331 RCTs	Depression	7 psychotherapy types	All effective; no between-therapy differences; combo > monotherapy	—

KEY NUMBERS TO MEMORISE

Number	What It Represents	Source
40/30/15/15	Outcome variance: client/relationship/expectancy/technique	Lambert (1992)
d = 0.80	Overall effect of psychotherapy vs no treatment	Wampold (2001)
r = 0.275	Alliance-outcome correlation	Horvath et al. (2011)
5:1	Positive-to-negative interaction ratio for stable couples	Gottman (1999)
45.5%	ST recovery rate for BPD	Giesen-Bloo (2006)
31% vs 76%	Relapse after prior CT vs medication withdrawal	Hollon (2005)
d = 1.30	EFT-C effect size	Johnson et al. (1999)
3+ episodes	MBCT relapse prevention threshold	Teasdale (2000)
93.6%	Gottman's divorce prediction accuracy	Gottman (1999)
13% vs 87%	BPD criteria at 8–year follow-up (MBT vs control)	Bateman & Fonagy (2008)

Weave Psychotherapy Vol. 12 — Landmark Papers & Trials

Dr. Wilfred Dsouza

02

Clinical Quick Reference

Landmark Papers & Trials — Weave Psychotherapy Vol. 12

D6: Clinical Quick Reference — Landmark Papers & Trials in Psychotherapy

1. Master Landmark Trials Table

Common Factors & Outcome Research

Trial	Year	Design	N	Key Finding
Luborsky et al.	1975	Narrative review	Multiple	Dodo bird verdict — no consistent differences between bona fide therapies
Lambert	1992	Review	—	Outcome variance: 40% client, 30% relationship, 15% expectancy, 15% technique
Wampold	2001	Meta-analysis	Multiple	Therapy vs no treatment d = 0.80; between-therapy differences d = 0.00–0.20
Wampold	2015	Meta-analysis	Multiple	Therapist effects (5–9%) > treatment differences; alliance r = 0.28

CBT Trials

Trial	Year	Design	N	Condition	Key Finding
Rush et al.	1977	RCT	41	Depression	First CT vs medication trial; CT superior to imipramine
Elkin et al. (TDCRP)	1989	Multi-site RCT	250	Depression	All active > placebo; CBT struggled in severe depression
DeRubeis et al.	2005	RCT	240	Moderate-severe MDD	CT = paroxetine (58% response each); refuted TDCRP
Hollon et al.	2005	Follow-up	104	Depression	Prior CT relapse 31% vs medication withdrawal 76%
Clark et al.	2003	RCT	60	Social anxiety	CT > fluoxetine > placebo; d > 1.0
Ehlers & Clark	2005	RCT	85	Chronic PTSD	CT d = 1.27 vs waitlist; 71% reliable change

DBT Trials

Trial	Year	Design	N	Condition	Key Finding
Linehan et al.	1991	RCT	44	BPD (women)	DBT reduced parasuicide + hospitalisations; NOT depression
Rathus & Miller	2002	Quasi-exp	111	Adolescent BPD	DBT-A reduced hospitalisations; 0 suicide attempts
Linehan et al.	2006	RCT	101	BPD (women)	DBT halved suicide attempts vs expert clinicians (23% vs 46%)

Psychodynamic Trials

Trial	Year	Design	N	Condition	Key Finding
Bateman & Fonagy	1999	RCT	38	BPD	MBT reduced self-harm, depression, hospitalisations
Bateman & Fonagy	2008	8–yr follow-up	38	BPD	13% still met BPD criteria vs 87% control
Clarkin et al.	2007	RCT	90	BPD	TFP improved mentalisation; DBT improved impulsivity
Leichsenring & Rabung	2008	Meta-analysis	1,053	Complex disorders	LTPP d = 0.96 for complex presentations
Shedler	2010	Meta-review	Multiple	Various	Psychodynamic d = 0.97; unique "sleeper effect"

Schema Therapy Trials

Trial	Year	Design	N	Condition	Key Finding
Giesen-Bloo et al.	2006	Multi-site RCT	86	BPD	ST recovery 45.5% vs TFP 24.4%; ST dropout 26.7% vs TFP 50.6%
Nadort et al.	2009	RCT	62	BPD	ST effective without phone crisis support
Bamelis et al.	2014	Multi-site RCT	323	Cluster C PD	ST recovery 81% vs COP 60% vs TAU 52%; largest PD trial

Trauma Trials (PE, CPT, EMDR)

Trial	Year	Design	N	Condition	Key Finding
Shapiro	1989	RCT	22	Trauma	Original EMDR study; heavily criticised but generative
Foa et al.	1999	RCT	96	PTSD (sexual assault)	PE alone best at follow-up; combined NOT superior
Resick et al.	2002	RCT	171	PTSD (sexual assault)	CPT = PE; both superior to control
Foa et al.	2005	RCT	179	PTSD	PE alone = PE + cognitive restructuring
Bisson et al.	2007	Cochrane review	38 RCTs	PTSD	TF-CBT and EMDR recommended; equivalent to each other

Family & Couples Therapy

Trial	Year	Design	N	Condition	Key Finding
Gottman	1999	Longitudinal	>3,000	Couples	Four Horsemen predict divorce (93.6%); 5:1 ratio
Johnson et al.	1999	Meta-analysis	7 studies	Couples	EFT-C d = 1.30; 70–73% recovery
Leff et al.	2000	Meta-analysis	Multiple	Schizophrenia	Family intervention halved relapse (~25% vs ~50%)

ACT & MBCT

Trial	Year	Design	N	Condition	Key Finding
Teasdale et al.	2000	RCT	145	Recurrent MDD	MBCT halved relapse for 3+ episodes (37% vs 66%)
Kuyken et al.	2015	RCT	424	Recurrent MDD	MBCT non-inferior to maintenance antidepressants over 24 months
A-Tjak et al.	2015	Meta-analysis	39 RCTs	Various	ACT g = 0.57 vs controls; equivalent to CBT (g = 0.06, ns)

Comparative & Process Research

Trial	Year	Design	N	Key Finding
Bordin	1979	Theory paper	—	Working alliance = tasks + goals + bond; r = 0.275 with outcome
Safran & Muran	2000	Process research	Multiple	Repaired ruptures predict better outcomes than no rupture
Kazdin	2007	Conceptual	—	Mediators are not mechanisms; five criteria for mechanism evidence
Cuijpers et al.	2008	Meta-analysis	53 RCTs	Depression: d = 0.67 (0.42 corrected for pub bias)
Steinert et al.	2017	Meta-analysis	23 RCTs	Psychodynamic vs CBT: g = -0.07 (no difference)
Cuijpers et al.	2019	Network MA	331 RCTs	All 7 therapy types effective for depression; combo > mono
Barlow et al.	2017	RCT	223	Unified Protocol non-inferior to disorder-specific CBT

2. Key Numbers to Memorise

Effect Sizes

Number	Meaning	Source
d = 0.80	Overall psychotherapy vs no treatment	Wampold (2001)
d = 0.00–0.20	Differences between bona fide therapies	Wampold (2001)
d = 0.67	Psychotherapy for depression (uncorrected)	Cuijpers (2008)
d = 0.42	Psychotherapy for depression (corrected for pub bias)	Cuijpers (2008)
d = 0.97	Psychodynamic therapy	Shedler (2010)
d = 0.96	LTPP for complex disorders	Leichsenring & Rabung (2008)
d = 1.30	EFT for couples	Johnson et al. (1999)
d > 1.0	Clark's CT for social anxiety	Clark et al. (2003)
d = 1.27	Clark's CT for PTSD	Ehlers & Clark (2005)
g = 0.57	ACT vs control conditions	A-Tjak et al. (2015)
g = -0.07	Psychodynamic vs CBT (no difference)	Steinert et al. (2017)
r = 0.275	Alliance-outcome correlation	Horvath et al. (2011)

Percentages and Ratios

Number	Meaning	Source
40/30/15/15	Outcome variance: client / relationship / expectancy / technique	Lambert (1992)
5–9%	Outcome variance explained by therapist effects	Wampold (2001)
7.5%	Outcome variance explained by alliance	Horvath et al. (2011)
5:1	Positive-to-negative ratio in stable couples	Gottman (1999)
93.6%	Gottman's divorce prediction accuracy	Gottman (1999)
45.5% vs 24.4%	ST vs TFP recovery in BPD	Giesen-Bloo (2006)
31% vs 76%	Relapse: prior CT vs medication withdrawal	Hollon et al. (2005)
23% vs 46%	Suicide attempts: DBT vs community experts	Linehan (2006)
37% vs 66%	Relapse: MBCT vs TAU (3+ episodes)	Teasdale (2000)
13% vs 87%	BPD criteria at 8–year follow-up: MBT vs control	Bateman & Fonagy (2008)
58% vs 58%	CT vs paroxetine response rate in moderate-severe MDD	DeRubeis (2005)
81%	ST recovery rate for Cluster C PDs	Bamelis (2014)
70–73%	EFT-C couple recovery rate	Johnson et al. (1999)

Number	Meaning	Source
50%	Relapse reduction from family intervention in schizophrenia	Leff et al. (2000)

3. "Who Proved What" Quick Reference

Depression

- **CBT works for depression:** Rush et al. (1977)
- **CBT works for severe depression:** DeRubeis et al. (2005)
- **CBT has enduring effects:** Hollon et al. (2005)
- **All therapies roughly equivalent:** Cuijpers et al. (2008, 2019)
- **MBCT prevents relapse (3+ episodes):** Teasdale et al. (2000)
- **MBCT = maintenance meds:** Kuyken et al. (2015)
- **Combo > monotherapy:** Cuijpers et al. (2019)

BPD

- **DBT reduces self-harm:** Linehan et al. (1991)
- **DBT beats expert treatment:** Linehan et al. (2006)
- **MBT works (longest follow-up):** Bateman & Fonagy (1999, 2008)
- **ST has highest recovery rate:** Giesen-Bloo et al. (2006)
- **TFP improves mentalisation:** Clarkin et al. (2007)
- **DBT-A works for adolescents:** Rathus & Miller (2002)

Personality Disorders (non-BPD)

- **ST works for Cluster C PDs:** Bamelis et al. (2014)
- **LTPP best for complex PDs:** Leichsenring & Rabung (2008)

PTSD

- **PE is the critical ingredient:** Foa et al. (1999, 2005)

- **CPT = PE:** Resick et al. (2002)
- **EMDR = TF-CBT:** Bisson et al. (2007)
- **Only TF approaches recommended:** Bisson et al. (2007)
- **Clark's CT produces largest effects:** Ehlers & Clark (2005)

Anxiety Disorders

- **CT superior to fluoxetine for social anxiety:** Clark et al. (2003)
- **Unified Protocol works transdiagnostically:** Barlow et al. (2017)
- **ACT works but = CBT:** A-Tjak et al. (2015)

Couples & Family

- **Four Horsemen predict divorce:** Gottman (1999)
- **EFT-C has largest effect size:** Johnson et al. (1999)
- **Family intervention halves schizophrenia relapse:** Leff et al. (2000)

Common Factors

- **Dodo bird verdict:** Luborsky et al. (1975)
- **Relationship > technique:** Lambert (1992), Wampold (2001)
- **Who delivers > what's delivered:** Wampold (2015)
- **Alliance predicts outcome:** Bordin (1979), Horvath et al. (2011)
- **Rupture-repair is therapeutic:** Safran & Muran (2000)

Psychodynamic Therapy

- **Evidence base is strong:** Shedler (2010)
- **LTPP for complexity:** Leichsenring & Rabung (2008)
- **Equivalent to CBT in direct comparison:** Steinert et al. (2017)

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4. Timeline of Psychotherapy Research Milestones

Year	Milestone	Significance
1975	Luborsky — Dodo bird verdict	Launched the common factors debate
1977	Rush et al. — CT vs imipramine	CBT entered the evidence arena
1979	Bordin — Working Alliance model	Pantheoretical alliance framework
1989	Elkin et al. — NIMH TDCRP	First large-scale psychotherapy horse race
1989	Shapiro — EMDR introduced	New trauma treatment paradigm
1991	Linehan — First DBT trial	BPD became treatable
1992	Lambert — 40/30/15/15 model	Defined outcome variance factors
1999	Bateman & Fonagy — MBT for BPD	Mentalisation-based treatment enters evidence
1999	Foa — PE for PTSD	Exposure confirmed as key ingredient
1999	Gottman — Divorce prediction	Couples research became empirical
1999	Johnson — EFT-C meta-analysis	Attachment-based couples therapy validated
2000	Teasdale — MBCT prevents relapse	Mindfulness entered evidence-based practice
2000	Safran & Muran — Rupture-repair	Alliance became a dynamic process, not a static trait
2001	Wampold — Great Psychotherapy Debate	Quantified common vs specific factors

Year	Milestone	Significance
2002	Resick — CPT = PE for PTSD	Two front-line trauma treatments established
2003	Clark — CT for social anxiety	Disorder-specific CBT benchmarks set
2005	DeRubeis — CT = medication for severe MDD	CBT's severe depression limitation refuted
2005	Hollon — CT's enduring effect	CBT protects after discontinuation
2006	Giesen-Bloo — ST vs TFP for BPD	Highest BPD recovery rate in any trial
2006	Linehan — DBT vs expert clinicians	DBT superiority confirmed against rigorous comparator
2007	Bisson — Cochrane PTSD review	TF-CBT and EMDR established as first-line
2007	Kazdin — Mechanisms of change	Shifted field from "does it work" to "how does it work"
2008	Leichsenring & Rabung — LTPP meta-analysis	Long-term psychodynamic therapy validated for complex disorders
2010	Shedler — Psychodynamic efficacy review	Psychodynamic therapy shown to match CBT effect sizes
2014	Bamelis — ST for Cluster C PDs	Largest personality disorder trial; ST efficacy broadened
2015	Kuyken — MBCT = maintenance meds	MBCT established as medication alternative
2015	A-Tjak — ACT meta-analysis	ACT validated; equivalent to CBT

Year	Milestone	Significance
2017	Barlow — Unified Protocol	Transdiagnostic CBT non-inferior to disorder-specific
2017	Steinert — Psychodynamic vs CBT	Direct comparison: no difference (g = -0.07)
2019	Cuijpers — Network meta-analysis	331 RCTs: all therapies work; combo best

5. Meta-Analyses Summary Table

Authors	Year	Focus	Studies	N	Main Result
Wampold	2001	Therapy vs no tx	Multiple	Multiple	d = 0.80
Cuijpers et al.	2008	Psychotherapy for depression	53 RCTs	>5,000	d = 0.67 (0.42 corrected)
Leichsenring & Rabung	2008	LTPP	23 studies	1,053	d = 0.96
Shedler	2010	Psychodynamic therapy	Multiple	Multiple	d = 0.97
Horvath et al.	2011	Alliance-outcome	>200 studies	>14,000	r = 0.275
A-Tjak et al.	2015	ACT	39 RCTs	>3,000	g = 0.57 vs controls
Steinert et al.	2017	PDT vs CBT	23 RCTs	>2,500	g = -0.07 (no difference)
Cuijpers et al.	2019	Psychotherapy for depression	331 RCTs	>34,000	All 7 types effective; combo > mono
Bisson et al.	2007	PTSD treatments	38 RCTs	>3,000	TF-CBT and EMDR first-line
Johnson et al.	1999	EFT-C	7 studies	>300	d = 1.30

6. Viva Questions

Q1. What is the Dodo bird verdict and who proposed it?

Luborsky et al. (1975). When bona fide psychotherapies are compared head-to-head, outcome differences are small or absent. Named after the Dodo bird in Alice in Wonderland: "everybody has won and all must have prizes." Supported by Wampold (2001): between-therapy differences d = 0.00–0.20.

Q2. What are Lambert's four factors of therapeutic outcome?

Extratherapeutic/client factors (40%), therapeutic relationship (30%), expectancy/placebo (15%), technique (15%). The relationship accounts for twice the variance of technique.

Q3. What is the strongest evidence that CBT works for severe depression?

DeRubeis et al. (2005): N = 240, moderate-to-severe MDD. CT and paroxetine had identical response rates (58%). This directly refuted the NIMH TDCRP (Elkin, 1989) finding that CBT was less effective for severe depression.

Q4. What evidence supports CBT's enduring effect?

Hollon et al. (2005): relapse rate after prior CT was 31% vs 76% after medication withdrawal. Prior CT was equivalent to continuing medication (47%). CBT provides lasting protection after discontinuation; medication does not.

Q5. Compare the evidence for DBT, MBT, ST, and TFP in BPD.

DBT (Linehan, 2006): halved suicide attempts vs expert treatment. MBT (Bateman & Fonagy, 2008): 8–year follow-up, 13% still met BPD criteria. ST (Giesen-Bloo, 2006): highest recovery rate (45.5%), lowest dropout. TFP (Clarkin, 2007): uniquely improved mentalisation. All effective; different mechanisms.

Q6. What is the evidence for MBCT in preventing depression relapse?

Teasdale (2000): MBCT halved relapse for patients with 3+ episodes (37% vs 66%). NOT effective for patients with only 2 episodes. Kuyken (2015): MBCT non-inferior to maintenance antidepressants over 24 months.

Q7. What are the recommended first-line treatments for PTSD?

Bisson et al. (2007, Cochrane): TF-CBT (including PE and CPT) and EMDR. Both are equivalent to each other. Non-trauma-focused therapies (supportive counselling, relaxation) are not recommended. Exposure is the critical ingredient (Foa, 1999, 2005).

Q8. What is the working alliance and how does it relate to outcome?

Bordin (1979): tasks, goals, bond. Alliance-outcome correlation r = 0.275 (~7.5% variance). Most replicated finding in psychotherapy research. Safran & Muran (2000): repaired alliance ruptures predict better outcomes than no rupture.

Q9. What did Kazdin (2007) argue about mechanisms of change?

Most claimed mechanisms are actually mediators. True mechanism evidence requires: temporal precedence, specificity, gradient, strong association, and plausibility. Very few psychotherapy mechanisms meet all criteria.

Q10. What is the evidence that psychodynamic therapy is effective?

Shedler (2010): d = 0.97, matching CBT benchmarks. Unique "sleeper effect" (continuing improvement post-treatment). Leichsenring & Rabung (2008): LTPP d = 0.96 for complex disorders. Steinert (2017): direct comparison with CBT shows g = -0.07 (no difference).

Q11. What is Gottman's contribution to couples therapy evidence?

Prospective longitudinal research with >3,000 couples. Four Horsemen (criticism, contempt, defensiveness, stonewalling) predict divorce at 93.6% accuracy. Stable couples: 5:1 positive-to-negative ratio during conflict. Contempt is the most toxic behaviour.

Q12. What is the largest psychotherapy trial for personality disorders?

Bamelis et al. (2014): N = 323. Schema therapy for Cluster C PDs (avoidant, dependent, OCPD) and PD NOS. ST recovery 81% vs COP 60% vs TAU 52%. ST was also more cost-effective.

EXAM STRATEGY

For viva, know one landmark trial per modality plus one meta-analysis. Minimum set: Rush (1977), Linehan (1991/2006), Bateman & Fonagy (1999), Giesen-Bloo (2006), Foa (1999), Teasdale (2000), Wampold (2001), Shedler (2010), Cuijpers (2019).

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